

BUSINESS & PRACTICE MANAGEMENT

Financial Policy, Good Faith Estimate & Superbill Pack

The patient financial agreement, the No Surprises Act estimate every self-pay patient is owed, a superbill template, and a fee-setting worksheet.

AUDIENCE

Psychologists · Psychiatrists

FORMAT

Forms + worksheet

VERSION

1.0 · Sept 2026

TIME TO ADAPT

1 hour

WHAT IT IS

A complete money toolkit for private and group practice: a plain-language financial policy, a Good Faith Estimate that meets the federal requirement for self-pay and uninsured patients, a superbill for out-of-network reimbursement, and a worksheet for setting fees.

USE IT WHEN

You are opening or changing a practice, adding self-pay patients, raising fees, or auditing how you handle cancellations and estimates.

CUSTOMIZE FIRST

Your fees, cancellation window, payer mix and contract terms, state rules on fees and collections, and your NPI, TIN and practice address.

TWO THINGS TO CHECK BEFORE YOU USE ANY OF THIS

First, payer contracts, Medicare and Medicaid rules often limit what you may charge for missed sessions, forms and balances, so your contracts come first. Second, the Good Faith Estimate applies to any patient who pays without using insurance, including patients who use a superbill to seek their own reimbursement. Many clinicians miss the second point.

Part A: Patient financial policy

This is written to be signed at intake alongside your informed consent. Keep it to one or two pages. A short policy that patients actually read prevents most billing disputes.

Financial policy for [Practice name]. This explains how fees, payment and insurance work. Please ask if anything is unclear.

Fees. My fees are: initial evaluation (about [60] minutes), [\$___]. Individual session (about [45] minutes), [\$___]. Extended session (about [60] minutes), [\$___]. Fees for letters, forms, phone calls beyond [10] minutes, records preparation and court-related work are charged at [\$___] per hour, and I will tell you in advance when a fee applies. I review my fees each year and will give you at least [30] days' notice of any change.

Payment. Payment is due at the time of service unless we agree otherwise in writing. I accept [card, HSA/FSA card, check]. A card is kept securely on file with your written permission and used only for fees under this policy.

Insurance. [I am in network with: ____] / [I am out of network. I will give you a monthly superbill you can submit to your plan. Reimbursement is between you and your plan, and I cannot guarantee it.] You are responsible for knowing your benefits, and for any copay, coinsurance, deductible or non-covered service. I will tell you if a claim is denied and we will talk about options.

Self-pay patients and Good Faith Estimates. If you do not have insurance, or choose not to use it, you have the right to a written Good Faith Estimate of expected charges before care begins. I will give you one, and I will give you a new one if your care plan or fees change.

Cancellations and missed sessions. Please give at least [24] hours' notice if you cannot attend. A session missed or cancelled with less notice is charged at [full fee / \$____], because that time is reserved for you. Insurance does not pay for missed sessions. I will make exceptions for emergencies, illness and unforeseen events, at my discretion. [In-network policy: I follow the cancellation rules in my payer contracts.]

Late arrivals. Sessions end at the scheduled time. If you arrive late, we will use the time that remains.

Returned payments and unpaid balances. A fee of [\$____] applies to a returned payment. If a balance is unpaid, I will contact you to talk about it. Before I involve a collection agency, I will give you [30] days' written notice, and I will share only the minimum information the law allows. I would much prefer to find a payment plan with you.

Reduced fees. [I offer a limited number of reduced-fee spaces. Ask me about them.] Reduced fees are reviewed [every six months].

Emergencies. I do not provide emergency services. In an emergency call 911 or 988.

Patient signature

Date

Clinician signature

Date

Part B: Good Faith Estimate (No Surprises Act)

Under the No Surprises Act, providers must give a written Good Faith Estimate of expected charges to any patient who is uninsured or who is not using insurance for the service. The rule is codified at 45 CFR 149.610. The estimate is not a contract, and the patient is not obliged to use your services.

Rule	What it requires
Who receives one	Uninsured patients and patients who pay out of pocket rather than through a plan, including patients who will seek their own reimbursement with a superbill. Patients enrolled in federal health programs such as Medicare and Medicaid are excluded.
When	For care scheduled at least ten business days ahead, within three business days of scheduling. For care scheduled three to nine business days ahead, within one business day. On request, before scheduling or at any time, within three business days of the request. No estimate is required for care scheduled less than three business days ahead.
Recurring care	One estimate can cover a series of recurring sessions, such as ongoing psychotherapy, for up to 12 months. Issue a new one when the period ends or the plan of care changes materially.
How	In writing, in plain language, in the format the patient chooses (paper or electronic) and in a form the patient can save.
Dispute rights	If the final bill is at least \$400 more than the estimate, the patient can start a federal patient-provider dispute resolution process within 120 days of the bill. You may not send the item to collections while a dispute is pending.
Records	Keep a copy and the date and method of delivery. Keep records for the period CMS requires (currently six years).

COPY THE OFFICIAL NOTICE WORDING

CMS requires specific notices about the estimate and dispute rights. Use the wording in CMS's sample estimate ([cms.gov/nosurprises](https://www.cms.gov/nosurprises)) rather than paraphrasing it in your own words. Store the exact text in your template so it stays consistent.

Good Faith Estimate template

Patient name	Date of birth	Date of estimate
Provider name and credentials	NPI	TIN
Practice address and place of service		
Service in plain language (for example, weekly individual psychotherapy)		First scheduled date, if any

Service	CPT code	Diagnosis code(s)	Expected number	Charge each	Expected total
Initial diagnostic evaluation	[90791]	[ICD-10-CM]	[1]	[\$]	[\$]
Individual psychotherapy, [45] minutes	[90834]	[ICD-10-CM]	[weekly x 52]	[\$]	[\$]

Service	CPT code	Diagnosis code(s)	Expected number	Charge each	Expected total
Estimated total (up to 12 months)					[\$]

Notices to include (use CMS wording). This is an estimate based on information known today. It does not include unexpected services. It is not a contract, and you are not required to receive these services from this provider. If the bill is at least \$400 more than this estimate, you can dispute it, and you can also ask me to update the bill, negotiate it or check for financial assistance. To learn how to start a dispute, go to cms.gov/nosurprises. Keep this estimate.

DIAGNOSIS CODES AND HONESTY

Use the diagnosis you reasonably expect at the time. If your assessment changes, update the estimate. Where the diagnosis is not yet known, use the code you reasonably expect from the referral or presentation and explain that it is provisional.

Part C: Superbill template

A superbill is an itemized receipt a patient can send to their own plan for out-of-network reimbursement. It must match your records exactly. It is a business document, so do not add clinical detail beyond what is needed for the claim.

Provider name, license and state		NPI	TIN
Practice name and address		Phone	
Patient name	Date of birth	Patient address	
Insurance company (if known)	Member ID	Group number	

Date of service	Place of service	CPT	Modifier	Diagnosis (ICD-10-CM)	Time	Fee	Paid

Provider signature	Date

Common psychotherapy and psychiatry codes

Verify against the current-year CPT manual and each payer's rules. Time thresholds below are the widely used ranges for psychotherapy codes.

Code	Service	Notes
90791	Psychiatric diagnostic evaluation (no medical services)	Typically psychologists, and other clinicians.

Code	Service	Notes
90792	Psychiatric diagnostic evaluation with medical services	Typically prescribers.
90832 / 90834 / 90837	Individual psychotherapy, about 30, 45 and 60 minutes	Roughly 16 to 37, 38 to 52 and 53 or more minutes of face-to-face time.
90833 / 90836 / 90838	Psychotherapy add-on to an E/M visit, about 30, 45 and 60 minutes	Used with an evaluation and management code by prescribers.
90839 / 90840	Psychotherapy for crisis, first 60 minutes, and each additional 30	Urgent assessment and crisis intervention.
90846 / 90847	Family psychotherapy without / with the patient present	Payer rules on who must be present vary.
90853	Group psychotherapy	Not for multiple-family group.
90785	Interactive complexity add-on	Use only when defined complicating factors are documented.
96130 / 96131 and 96136 to 96139	Psychological testing evaluation and test administration	Follow the current-year rules on time and who performs services.
Telehealth	Place of service 10 (patient's home) or 02 (other location); modifier 95 (audio-video) or 93 (audio-only)	Payers differ. Confirm each payer's required place of service and modifier.

Part D: Fee-setting worksheet

Set fees from what your practice needs, not from what feels comfortable. Complete once a year. The example is fictional.

Line	Your figure	Example
A. Target annual take-home income before tax		\$150,000
B. Annual practice expenses (rent, software, insurance, billing, CE, licenses, marketing, accounting)		\$30,000
C. Annual revenue needed (A + B, plus about 25% for taxes, benefits and retirement, if you self-fund them)		\$225,000
D. Weeks worked per year (subtract vacation, holidays, CE, sick time)		46
E. Expected paid sessions per week (allow for cancellations, no-shows and admin time)		22
F. Paid sessions per year (D x E)		1,012
G. Break-even average fee per session (C / F)		\$222
H. Compare with your contracted and self-pay rates. What is the gap?		

Line	Your figure	Example
I. Decisions: raise self-pay fee, change payer mix, reduce hours, or cut expenses		

IN PSYALLIANCE

Share your anonymized fee-setting worksheet and cancellation policy wording in the World library so colleagues in your specialism and state can compare. Keep patient details out of anything you upload.

Sources and further reading

- HHS/CMS, No Surprises Act: Good Faith Estimates and Patient-Provider Dispute Resolution, 45 CFR 149.610 ([cms.gov/nosurprises](https://www.cms.gov/nosurprises)).
- CMS, sample Good Faith Estimate for uninsured or self-pay individuals ([cms.gov](https://www.cms.gov)).
- AMA, CPT psychotherapy and psychiatric diagnostic codes 90791 to 90853 (current-year manual).
- APA Ethics Code, Standards 6.04 (fees and financial arrangements) and 6.05 to 6.06 (bartering, accuracy in reports to payors) ([apa.org/ethics/code](https://www.apa.org/ethics/code)).

ABOUT THIS DOCUMENT

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